



February 23, 2026

Subject: Response to Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (HHS-ONC-2026-0001-0001)

To Secretary Robert F Kennedy, Jr., U.S. Department of Health and Human Services (HHS), and Steven Posnack, Principal Deputy Assistant Secretary for Technology Policy (ASTP):

The [Digital Medicine Society \(DiMe\)](https://www.dimesociety.org) is a global non-profit dedicated to advancing the safe, effective, and equitable use of digital approaches to redefine healthcare and improve lives. We appreciate the opportunity to respond to this Request for Information (RFI) on accelerating the adoption and use of artificial intelligence (AI) in clinical care.

DiMe convenes the ecosystem directly responsible for operationalizing AI adoption: developers, health systems, regulators, policymakers, payors, and patients. Our response focuses on practical barriers and policy levers identified by our team and community, emphasizing alignment across regulation, payment, governance, interoperability, and workforce readiness.

1. Barriers to AI Innovation and Adoption

The most significant barriers to private sector innovation and adoption of AI in clinical care are economic, operational, and governance-related.

- **Payment and revenue friction:** Startups lack early revenue pathways to generate sustainable income prior to broad reimbursement. Fee-for-service payment structures reward clinician time and service volume rather than efficiency, automation, or measurable outcome improvement.
- **Regulatory and governance uncertainty:** Developers face unpredictability in FDA review timelines and expectations amidst evolving guidance, particularly when determining whether tools meet the definition of a medical device. Health systems and clinicians also face challenges in determining whether AI outputs are reliable, valid, and appropriate for their patient populations.
- **Health system procurement and workflow integration:** Selling into hospital systems remains difficult due to point-solution fatigue, data and platform fragmentation, and heterogeneous privacy, security, and legal review processes. Integration into existing workflows, particularly alignment across IT infrastructure and governance guardrails, is disparate and resource-intensive.



Institutional friction and misaligned incentives, rather than technical capability, remain the primary constraints on adoption of rapidly advancing clinical AI systems.

2. Priority Regulatory and Payment Changes

HHS should prioritize regulatory and payment coordination that reflects the realities of adaptive AI systems and evolving clinical use cases.

- Support coordinated and redesigned payment models:** The collaboration between U.S. Food and Drug Administration's (FDA) TEMPO program and Centers for Medicare & Medicaid Services' (CMS) ACCESS initiative shows how coordinated regulatory and payment structures could support adaptive oversight and reimbursement redesign. CMS can further use existing payment authorities (e.g., 42 CFR § 410.26; 42 CFR § 414.40) and value-based program mechanisms (e.g., 42 CFR Part 414, Subpart O) to incentivize adoption of AI in high-burden, undermeasured conditions, including [women's health](#).
- Illustrate clearer boundaries through use cases:** The ecosystem requires greater insight into how models are trained and why certain AI products do or do not fall under FDA oversight as medical devices. Recent examples, such as [Doctronic's](#) AI-supported prescription renewal, illustrate the need for transparent, use-case-driven explanations so developers and health systems can design and implement responsibly within defined regulatory guardrails.
- Enable adaptive oversight pathways:** Where strong post-market monitoring, documentation, and performance dashboards are in place, HHS should consider mechanisms that allow expanded AI functionality or streamlined review processes without requiring duplicative submissions. The recent citizen petition requesting exemption from certain premarket notification requirements ([FDA-2025-P-5560](#)) illustrates industry interest in tying regulatory flexibility to demonstrable lifecycle monitoring capability.

Regulatory clarity, payment redesign, and cross-agency alignment will reduce uncertainty and accelerate responsible AI adoption.

3. Legal and Implementation Challenges

Non-device AI tools raise governance and accountability challenges that existing frameworks do not fully address.

- Training on proportional risk:** The updated [clinical decision support system \(CDSS\) guidance](#), emphasizing transparency and interpretability, introduces a need to train clinicians to review AI outputs relative to the strength of evidence and potential clinical impact.



- **Integrating data from non-devices:** The updated [general wellness guidance](#), focused on physiologic parameter outputs, may result in disparate forms of clinical measures being integrated into the EHR. When these data influence clinical workflows, governance mechanisms must ensure the integrity, auditability, accountability, and appropriate use of all inputs to clinical decision-making.
- **Fragmented frameworks:** Scaling innovative technology is slowed by disparate privacy, security, and legal frameworks across health systems. Establishing a harmonized baseline would reduce duplicative review processes that delay adoption, such as by endorsing pathways like the [DiMe Seal](#), which was developed to create a consistent benchmark for evidence, privacy, security, and usability for everyone adopting digital health software products.

HHS can play a coordinating role in clarifying and aligning how these expectations function across agencies.

4. AI Evaluation Methods for Clinical Care

Effective evaluation requires both structured pre-deployment benchmarking and disciplined post-deployment monitoring.

- **Pre-deployment benchmarking:** AI tools should be evaluated against defined performance metrics (e.g., drift and bias), with clear documentation artifacts describing intended use, limitations, and validation methods. DiMe's [The Playbook: Implementing AI in Healthcare](#) includes a section detailing relevant metrics such as defined KPIs and measures of success.
- **Post-deployment monitoring:** Live tools should undergo regular evaluation against those benchmarked metrics, supported by performance dashboards that enable human oversight and, as capabilities continue to advance, agentic supervisory approaches to detect drift or degradation across deployments (e.g., "AI monitoring AI"). [The Playbook](#) includes information on a minimum monitoring stack and governance best practices, and DiMe is committed to building upon this foundation with our upcoming project focused on [operationalizing AI governance in healthcare](#).
- **Ecosystem-level learning:** Rather than mandating a single harmonized evaluation process, HHS should support mechanisms that enable transparent sharing of implementation lessons, including case study libraries and federated sandbox environments that capture both successes (e.g., improved outcomes) and failures (e.g., evidence of harm).



HHS can lead on enabling shared learning from testing and evaluation to strengthen accountability while preserving flexibility for AI innovation.

5. Private-sector Activities HHS Can Support

HHS can support private-sector activities that strengthen trust and reduce duplicative oversight burdens for AI-enabled healthcare.

- Integration of comprehensive frameworks:** Recognizing comprehensive frameworks of standards and best practices, such as the [DiMe Seal](#), will build trust in the market, accelerate the adoption of digital health tools to improve health outcomes, and invite a continuous cycle of rigorous evaluation to elevate the practices of digital health. HHS should explore pathways to incorporating these frameworks into federal programs (e.g., Medicare App Library) where appropriate.
- Streamlined regulatory flexibility mechanisms:** While the FDA's precertification pilot program did not achieve its intended outcomes, alternative approaches to regulatory flexibility, such as mechanisms contemplated in the citizen petition ([FDA-2025-P-5560](#)), should be further considered, as they may offer more targeted pathways to streamline oversight supported by robust lifecycle monitoring.

Together, these activities can promote innovative and effective AI use in clinical care while reducing duplicative review and strengthening accountability.

6. Where AI Has Met Expectations or More Evidence is Needed

In certain applications that directly impact clinical operations, care providers, and patient experience, AI tools are already meeting or exceeding expectations. Where evidence is variable or limited, impact will depend on integration and monitoring.

- Surgical scheduling:** AI to predict accurate [surgical case duration](#) has demonstrated improved surgical suite utilization and margins.
- Automated triage:** AI to expedite [psychiatric triaging](#) promises to address long wait times for mental health care by facilitating more timely, potentially lifesaving care via improved intake in behavioral health settings.
- Ambient documentation:** Peer-reviewed [literature](#) reports reduced documentation burden and improved clinician experience following ambient AI scribe deployment, while realized productivity and financial impact will depend heavily on workflow integration and performance monitoring.

Comprehensive monitoring as part of operationalized governance will further illuminate where and how AI makes the most impact in clinical care settings.



7. Decision-makers and Administrative Barriers

Adoption of AI in clinical care is shaped by internal operational decision-making structures, including people, processes, and technologies.

- **Roles and decision-makers:** IT departments, formal or informal AI governance committees (where they exist), and individuals responsible for workforce training and education play central roles in determining whether and how AI tools are safely adopted.
- **Administrative hurdles:** Primary administrative barriers include lack of operationalized governance processes, point-solution fatigue, workflow integration challenges, and limited access to an ecosystem of tools that meet harmonized privacy, security, and integration requirements and can be deployed quickly with robust post-deployment monitoring.
- **Supporting resources:** Efforts including DiMe's upcoming [Operationalizing AI Governance in Healthcare](#) project, the governance section of [The Playbook: Implementing AI in Healthcare](#), and the [DiMe Seal](#) provide practical support for organizations implementing consistent oversight, accelerating responsible adoption across the ecosystem by building a strong foundation of trust.

Clarifying roles, reducing administrative friction, and equipping organizations with practical evaluation frameworks and governance tools will be essential to scaling clinical AI adoption responsibly.

8. Interoperability and Data Standards

Clarifying what interoperability is intended to achieve, and supporting it with unified code systems and documentation artifacts, will help developers avoid unnecessary implementation hurdles.

- **Interoperability beyond EHR integration:** Interoperability should foremost enable timely and safe access to actionable information for clinical users. Rather than requiring full archival integration into the EHR, human-centered dashboards supported by standardized API access can provide a faster and safer pathway to AI implementation. API availability should support third-party integration without creating unnecessary barriers to health system adoption.
- **Code system expansion:** The Veterans' Administration (VA) developed a [patient-generated health data \(PGHD\) code system](#), demonstrating how structured integration pathways can support implementation within a defined ecosystem. However, approaches that are not industry-wide limit broader development, interoperability, and adoption across systems and care settings.



- **Regulatory documentation artifacts:** AI-enabled medical device and sensor-based digital health technology (sDHT) developers can provide standardized documentation for regulatory submissions to streamline both health system and FDA review. Consistent documentation expectations have been articulated in recent [final guidance](#), creating a pathway for developers to avoid duplicative review and accelerate responsible deployment.

User-centered interoperability strategies paired with standardized code systems and documentation expectations will widen market access while preserving safety and regulatory integrity in AI-enabled care environments.

9. Patient and Caregiver Challenges

Patient and caregiver AI challenges stem from broad healthcare digitization and workflow gaps, not resistance to the technology.

- **Outdated workflows and misaligned expectations:** Both patients and clinicians experience friction when AI is introduced into legacy workflows. Patients facing long wait times for appointments often turn to AI to research symptoms or interpret results, while physicians express concerns about misinformation with limited time to address AI outputs within existing reimbursement structures. Introducing AI into outdated workflows without redesign can exacerbate tension rather than improve care delivery.
- **Patient education and records access:** Structured education on responsible AI use can improve clinical encounters and reduce confusion. Educational priorities include how to securely and efficiently access medical records, how to interpret AI-generated outputs cautiously, and how to prepare informed questions for appointments. Modernizing patient-facing reports (e.g., imaging, pathology, and laboratory results) to be clearer and more structured may also reduce reliance on AI interpretation and limit avoidable prompting errors.
- **Clinician training and trust-building:** Clinicians require training to engage constructively with patients who use AI tools, including how to discuss AI-generated insights transparently and respectfully. Health systems can support communication and trust by deploying AI applications that are both patient- and provider-facing, ensuring shared visibility into tools informing care.
- **Supporting resources:** DiMe is launching the [Scaling Trusted, High-Impact AI Care Navigation](#) project, focused on care journeys that identify patient and caregiver friction points and opportunities for AI-enabled workflows that demonstrate trust, utility, and impact, including return on investment (ROI).



Digitizing the ecosystem, redesigning AI-enabled workflows and care journeys, and educating patients, caregivers, and clinicians are essential to realizing the benefits of AI-enabled care while preserving trust.

10. Priority Areas for AI Research

Targeted research investments can accelerate adoption by clarifying economic value, clinical impact, and the infrastructure required to sustain responsible AI deployment.

- ROI models:** Research on ROI models for AI tools can clarify how value is created and distributed across stakeholders, supporting more informed adoption and payment decisions for both established and emerging use cases.
- Clinical impact studies:** Research comparing AI-enabled outcomes with standard human-led care processes, such as [this study](#) demonstrating consistently favorable outcomes via AI-supported mammography screening, provide the evidence of clinical impact needed to spur ecosystem adoption.
- Supporting resources:** DiME is committed to convening the digital healthcare ecosystem in pursuit of answering these high-priority research areas. For example, our upcoming project [Scaling Trusted, High-Impact AI Care Navigation](#) will produce a trust-utility-impact framework to classify which use clinical care use cases are ready for AI enablement, alongside evidence-informed ROI models estimating value in terms of outcomes, cost savings, and retention.

Prioritizing research that connects clinical performance and economic impact will support responsible scaling of AI in clinical care.

In Closing

DiME appreciates the opportunity to contribute to this RFI. Accelerating AI adoption in clinical care will require coordinated regulatory clarity, payment reform, interoperability and governance infrastructure, and workforce readiness. We welcome continued collaboration between our community and HHS to support practical, scalable, and trustworthy AI implementation across the healthcare ecosystem.

Sincerely,

Signed by:

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Benjamin Vandendriessche, PhD
President & Chief Scientific Officer

DocuSigned by:

 33CE75AC19204E9...

Jennifer Goldsack, MChem, MA, MBA, OLY
Chief Executive Officer